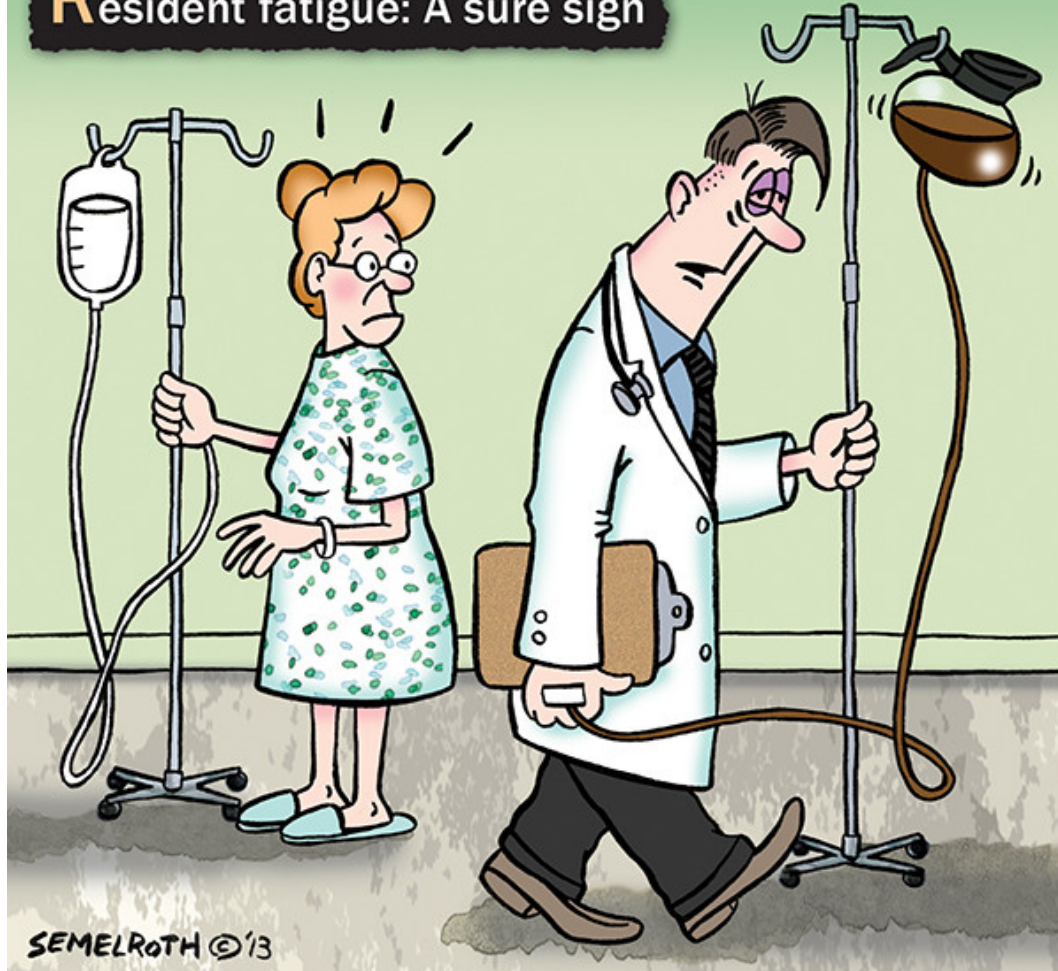


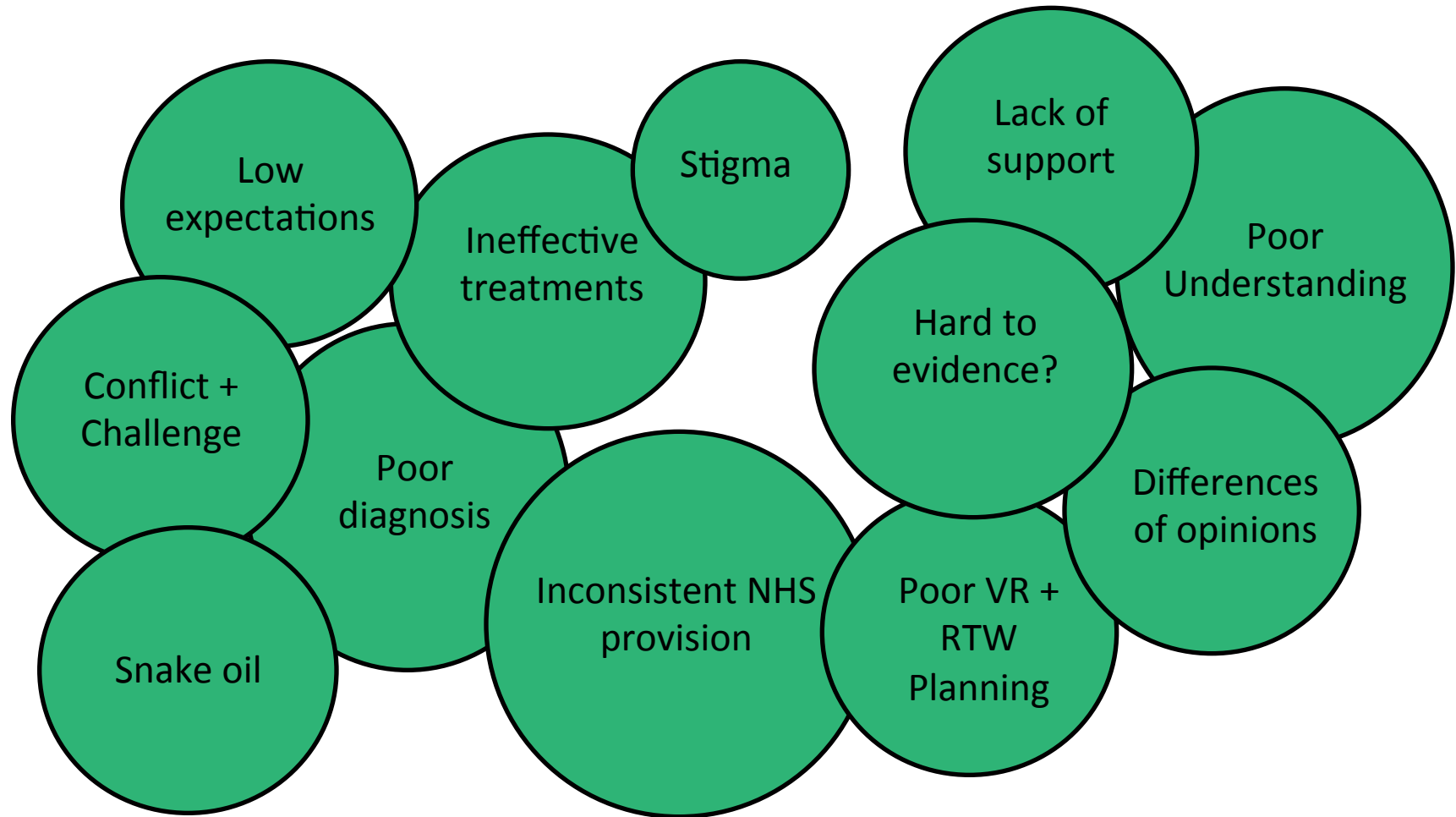
Resident fatigue: A sure sign

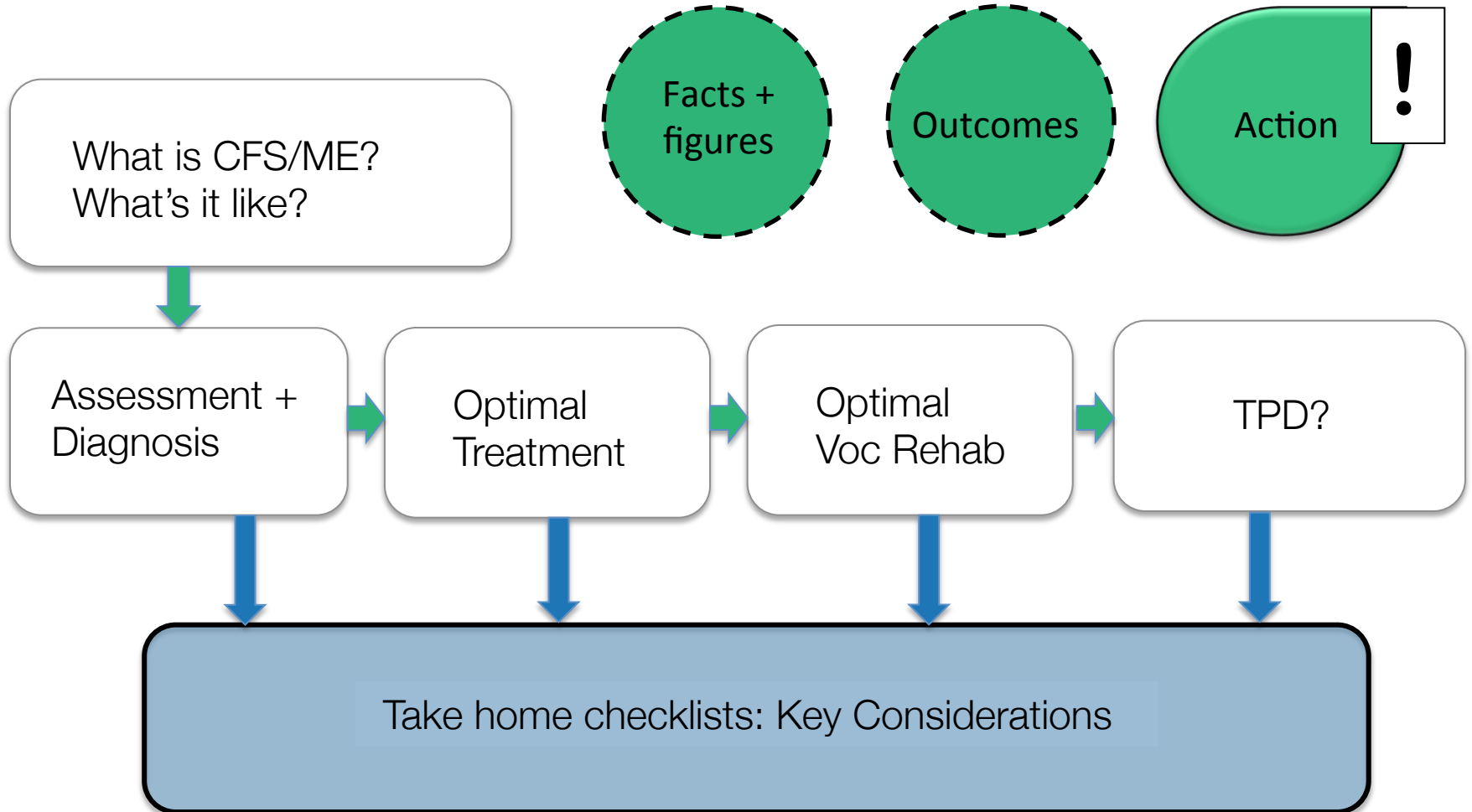


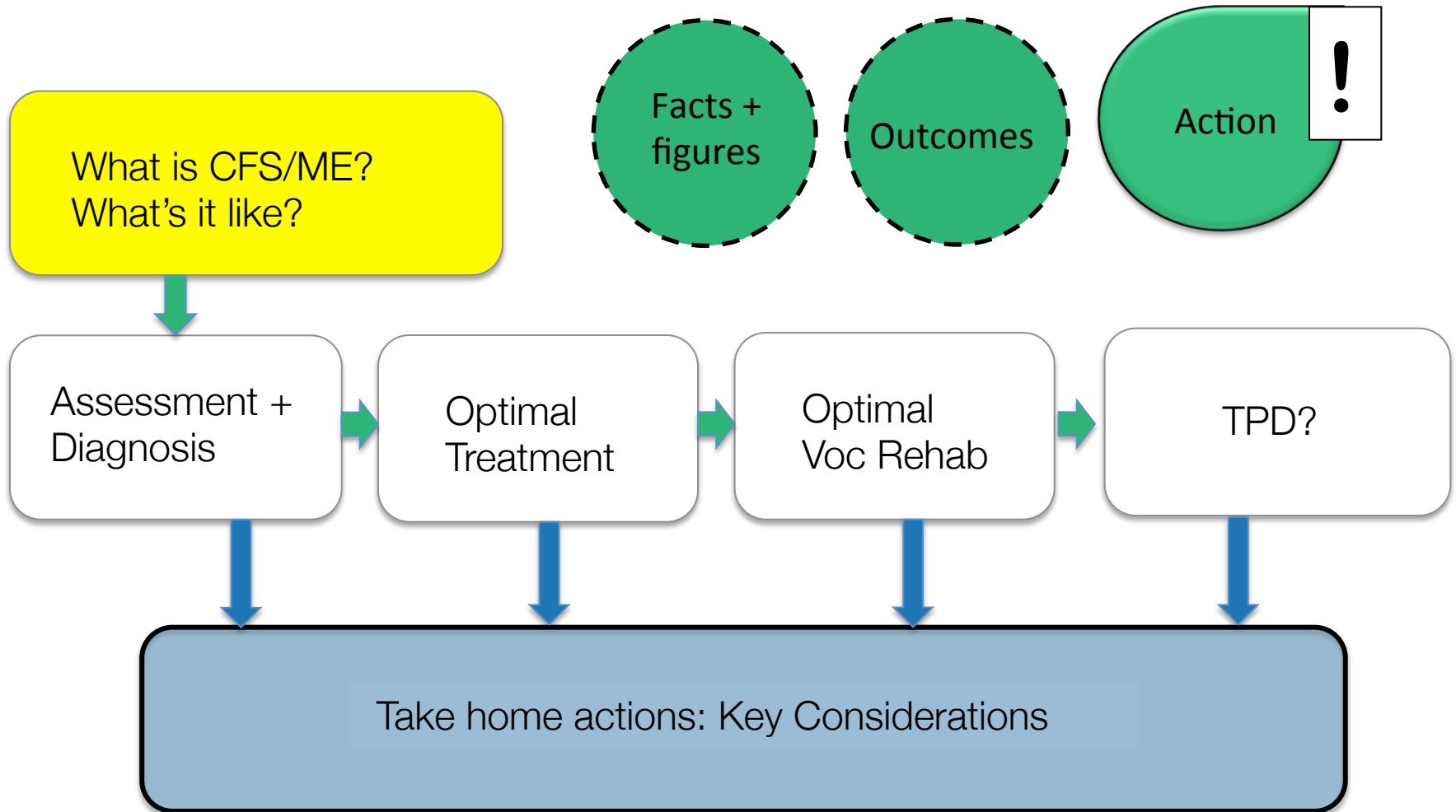
Demystifying CFS/ME

Health Claims
Forum
22 May 2014

CFS/ME: A complex interplay







Chronic fatigue

Persistent Fatigue

Idiopathic Fatigue



62% respondents
found it difficult to
understand CFS/ME

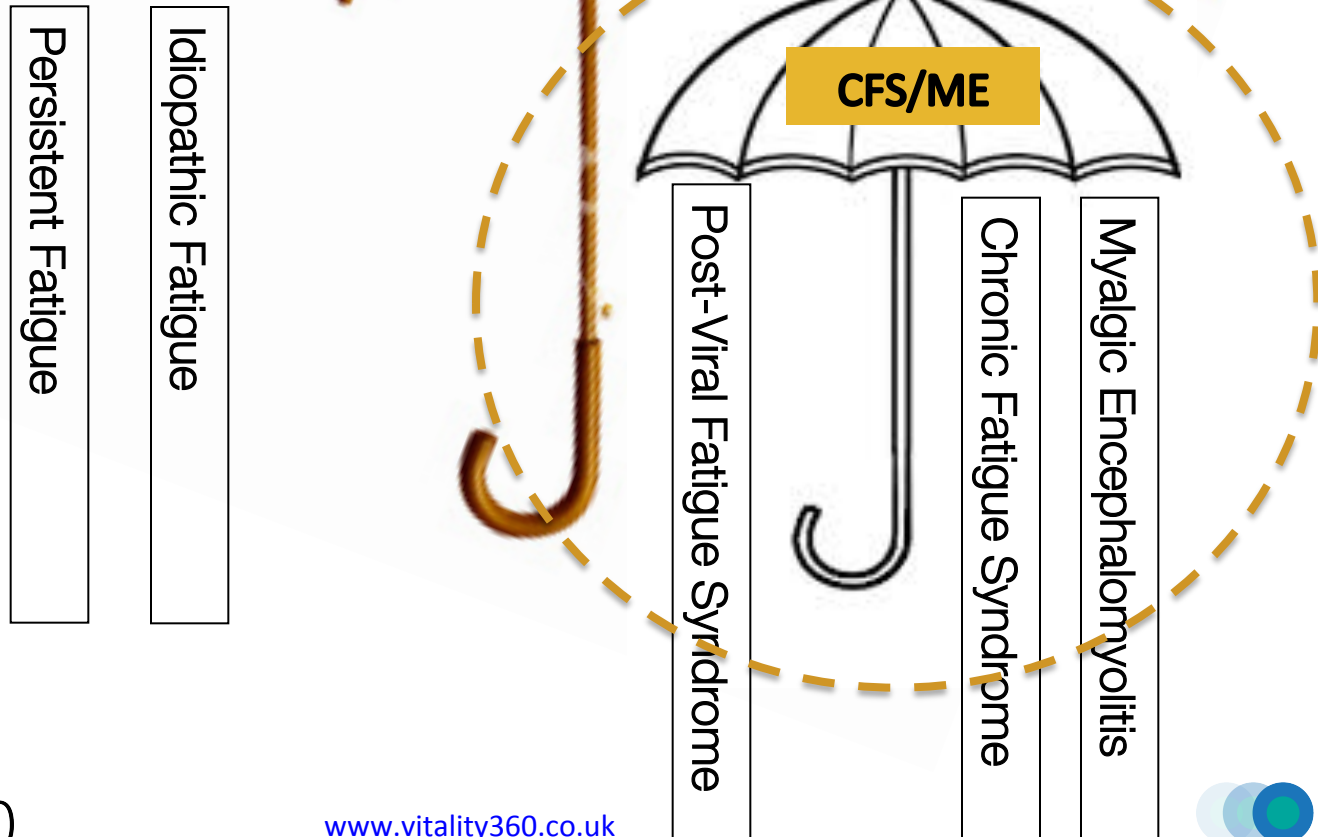


What is chronic fatigue?

- *“Physical or mental exhaustion or extreme tiredness due to activity, effort or illness, persisting for a long time or recurring.”*
 - Someone going through a hard time emotionally? Divorce, bullying, stress?
 - Impact of running a marathon? Pushing too hard?
 - Friday nights or weekends after a long working week?
 - Students at exam times? Pressures and deadlines?
 - Taking on a new physical or mental challenge?
 - Feeling exhausted after a bout of the ‘flu?



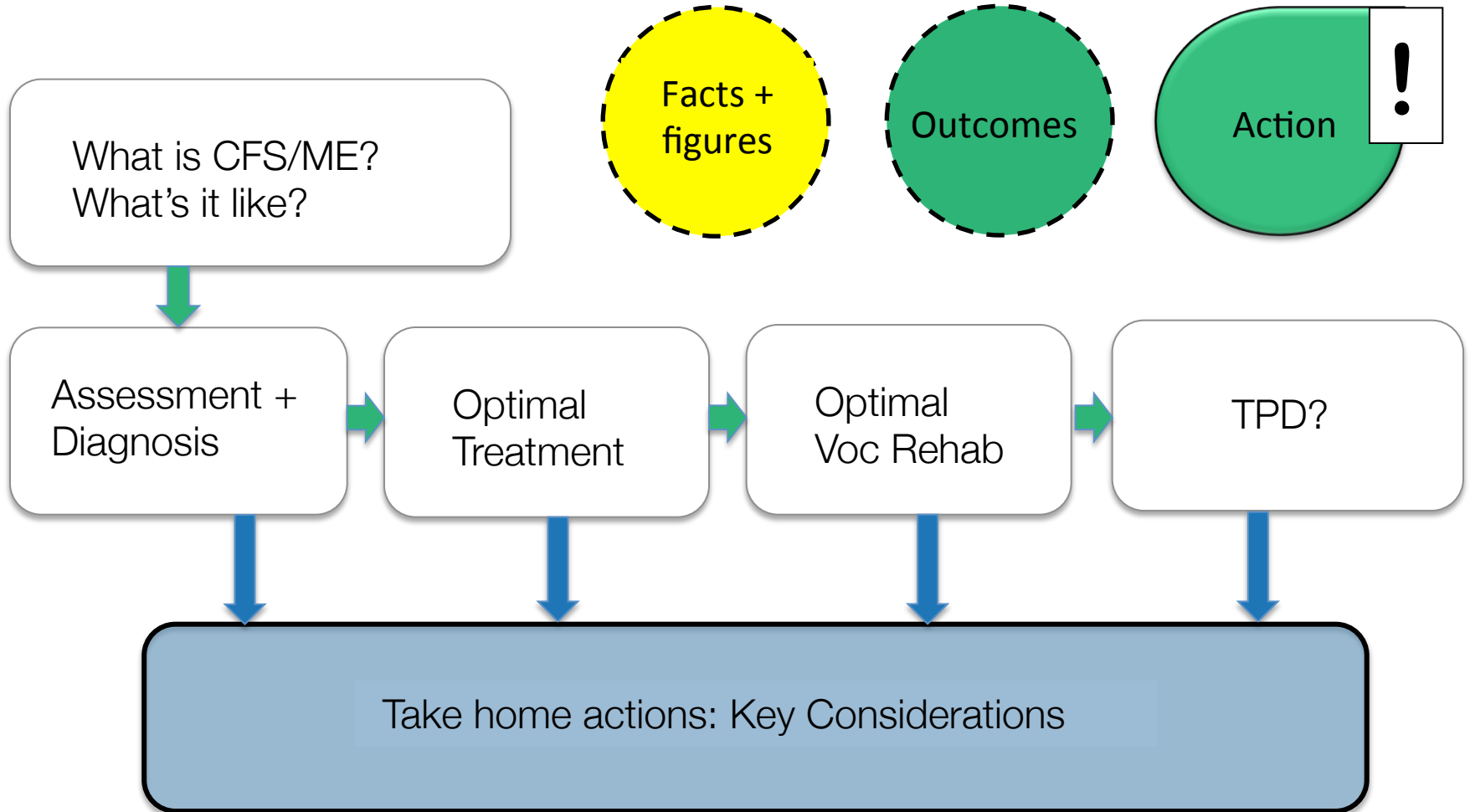
Chronic fatigue



What is the impact of CFS/ME on everyday life?

A short video as used in our VRA Award-Winning Inspiring People Project





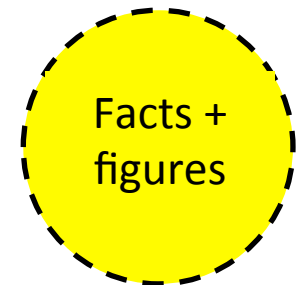
Fatigue Stats:

Prevalence:

- 'Persistent fatigue': 3-5% of population (new cases per year)
- CFS/ME: 0.2-0.4% of population, ie 200-400 per 100,000 people

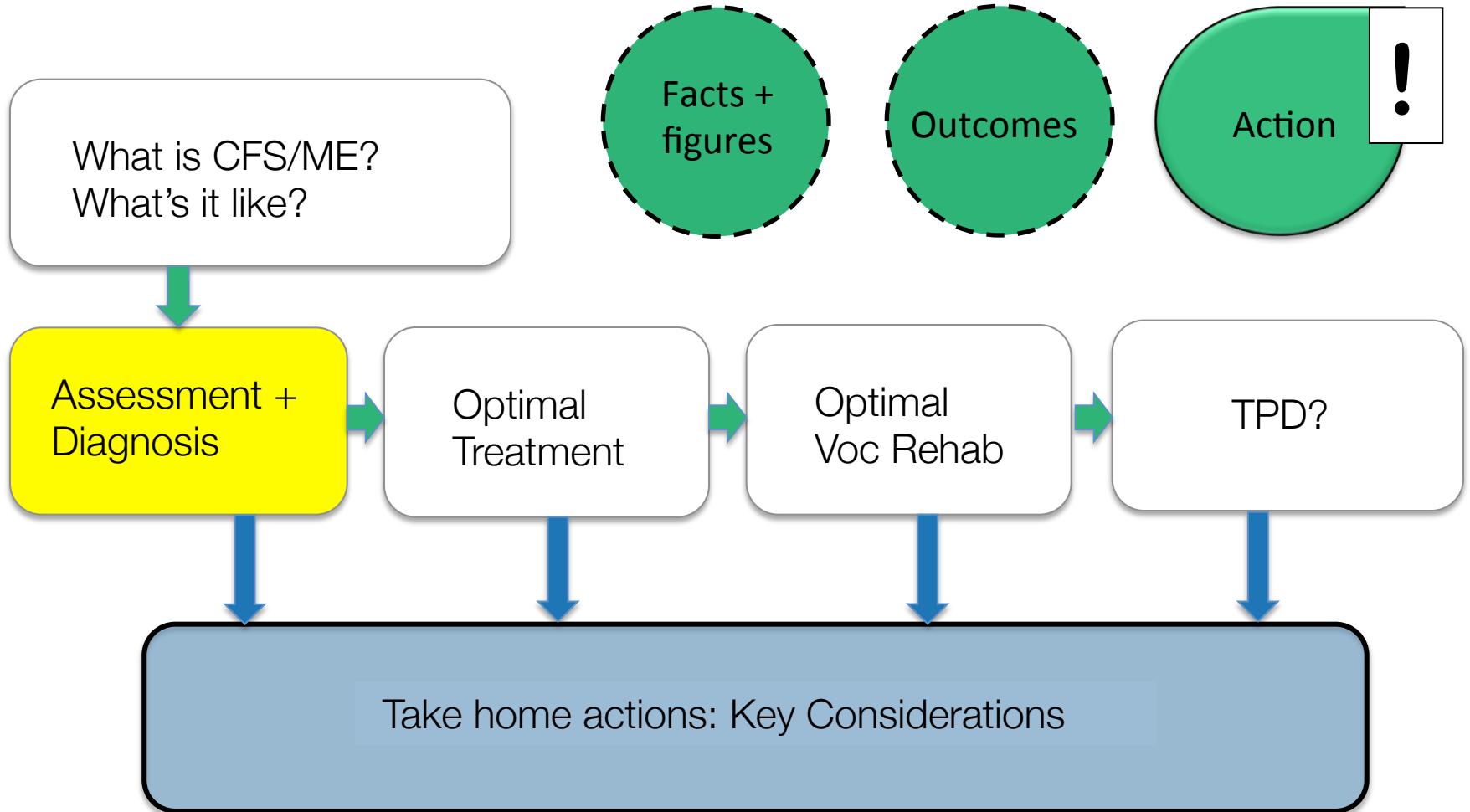
Approx 2-3 times more females to males (2/3:1 ratio)

Predominance of working age adults: aged 20-40

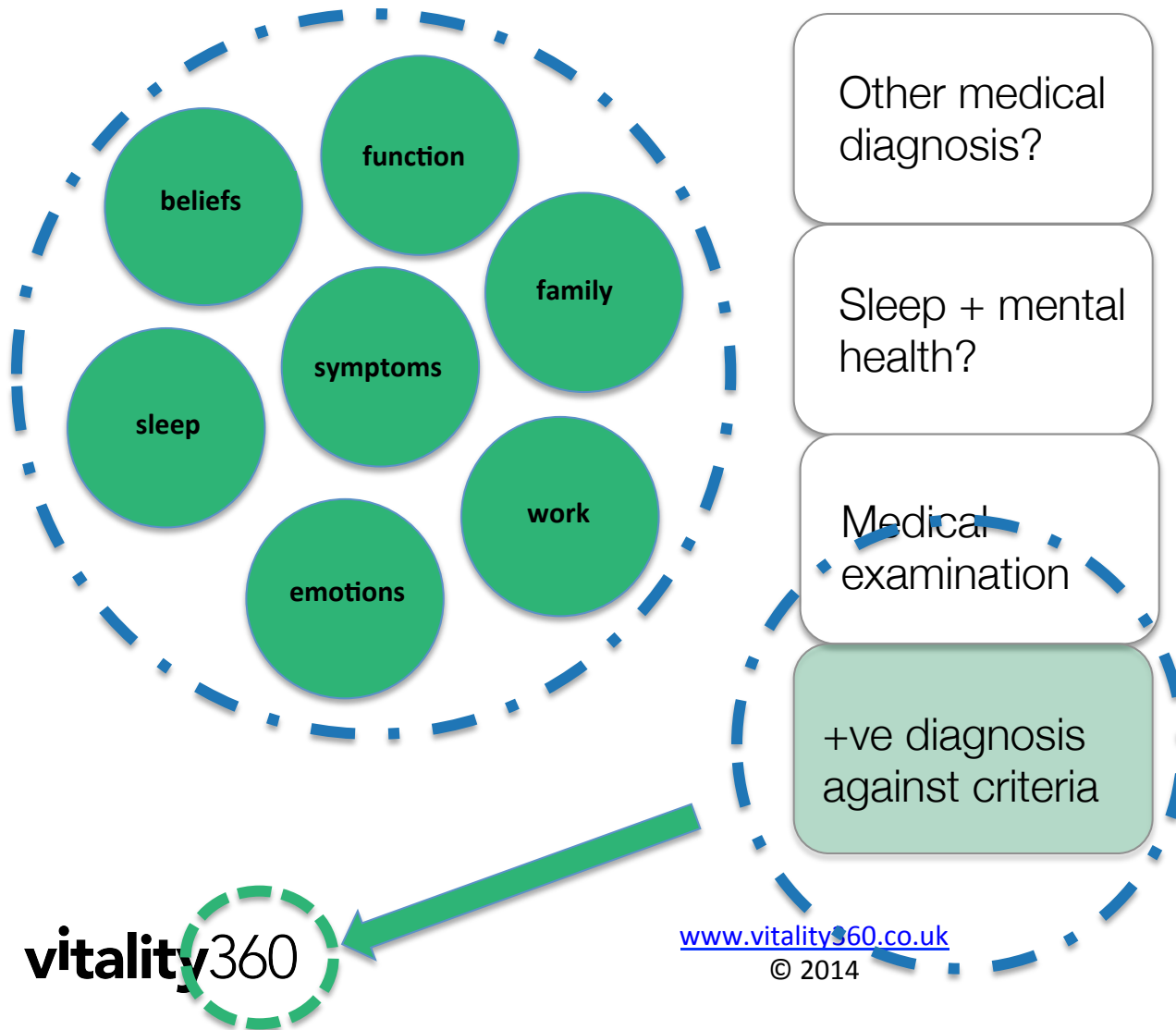


Ref: NICE Guidelines, CFS/ME Working Group





360 BPS Assessment:



- Is work important?
- Motivation?
- Beliefs about illness + recovery
- Life events + impact
- Previous experience
- Confidence
- What do colleagues think?
- Self-efficacy
- Context of their life
- Ambitions + Goals
- What have they heard?
- What's getting in the way?
- Are they being believed?



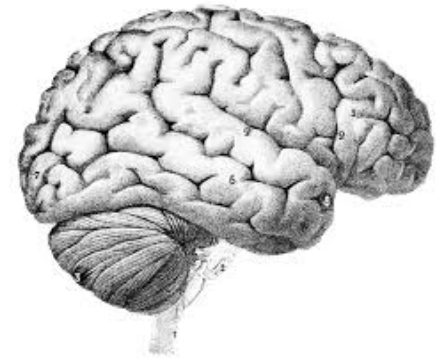
How is CFS/ME diagnosed?



6 months duration



Substantial impact + disruption:
More than 50% of the time

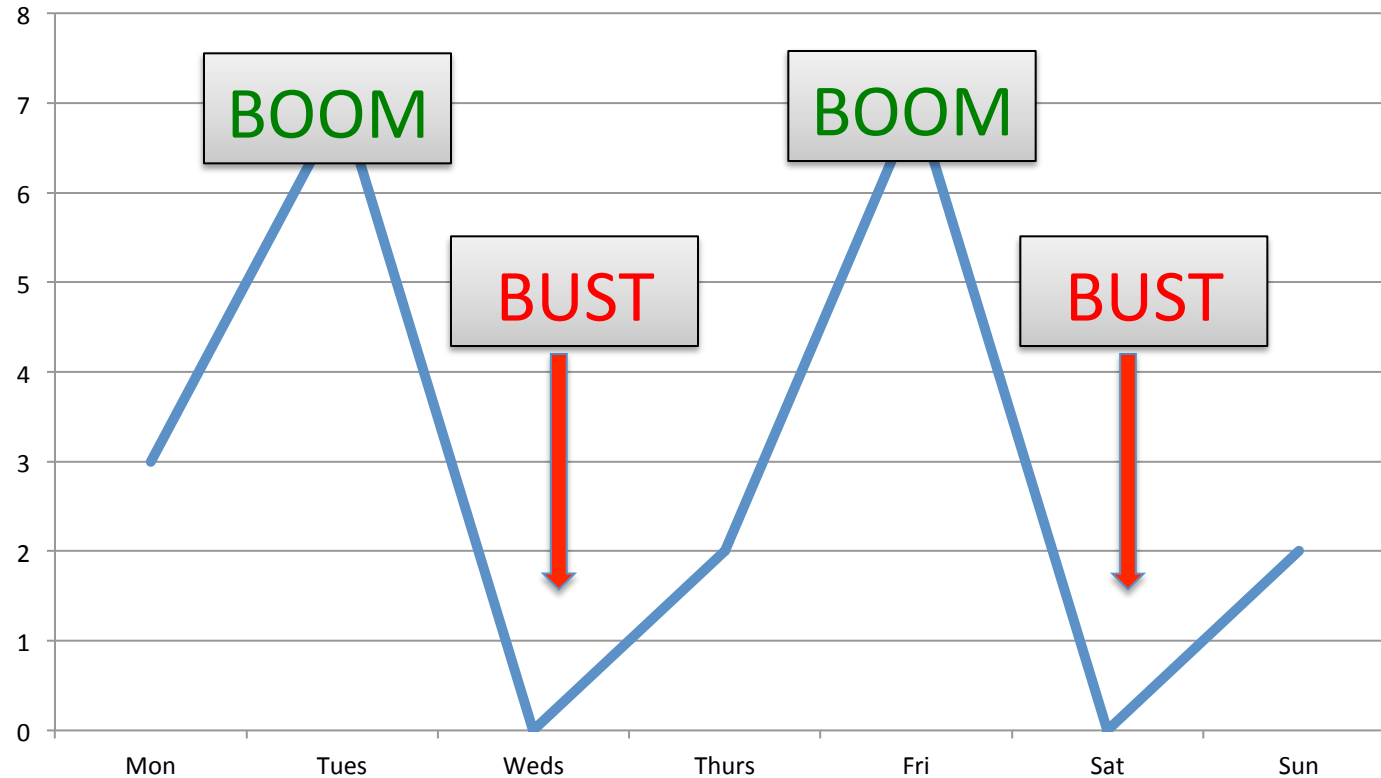


Cognitive +
neuropsychiatric
symptoms

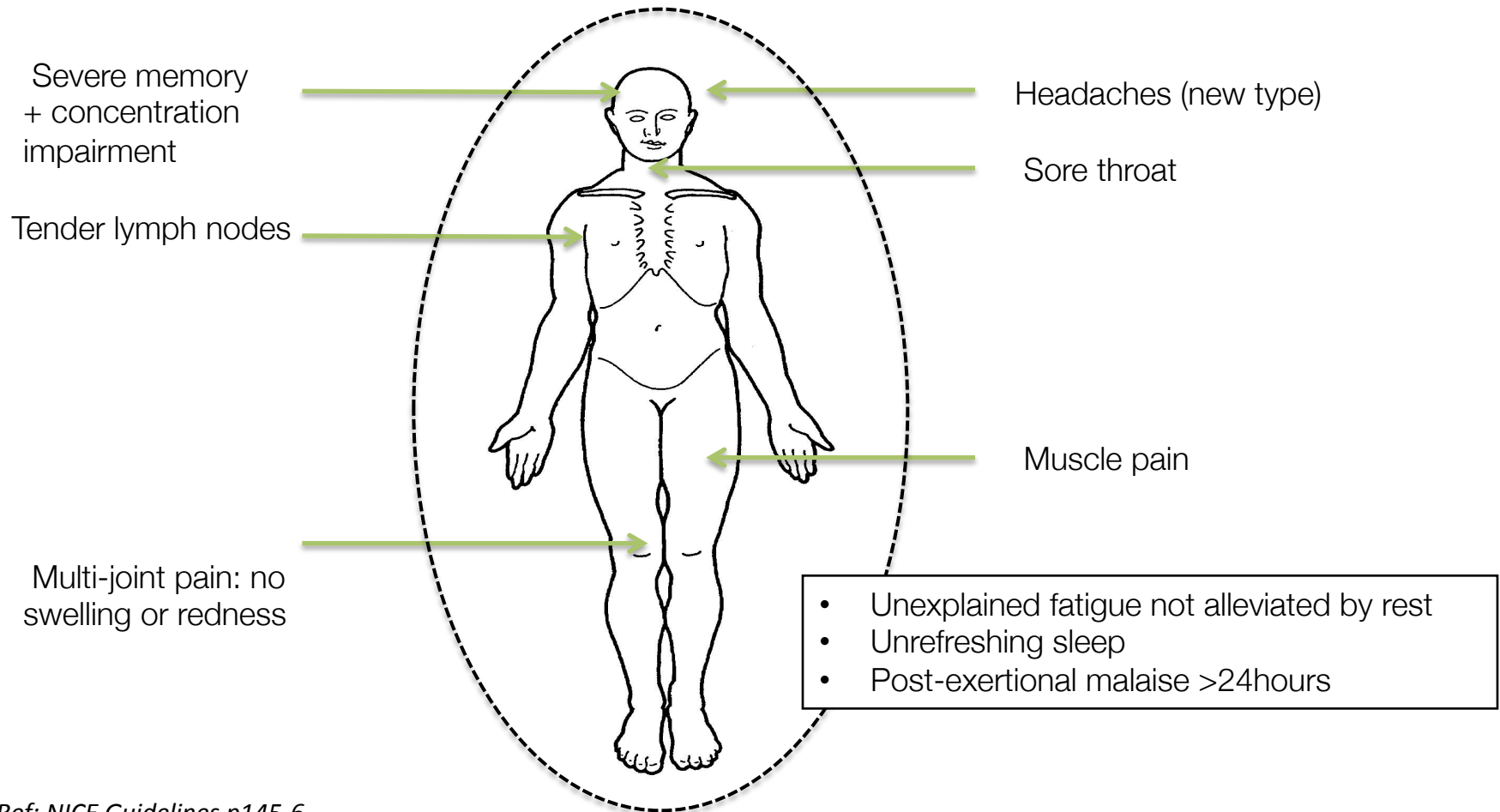


How is CFS/ME diagnosed?

Post-exertional fatigue ('boom-bust')



Diagnostic symptoms:



Ref: NICE Guidelines p145-6



Common additional symptoms:

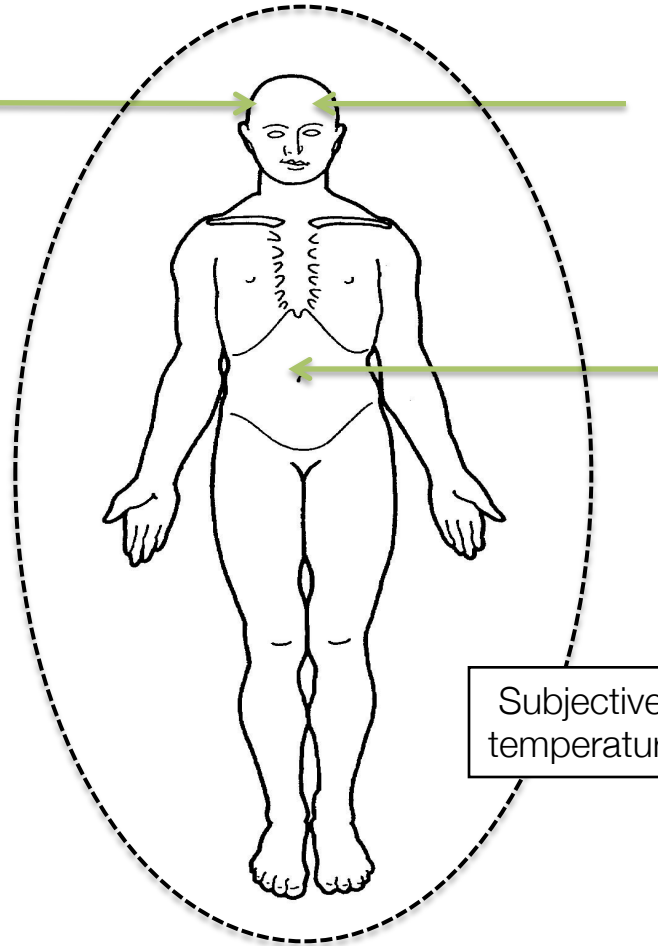
Mood disturbance,
irritability

Hypersensitivity (noise, sound)

NB: As distinct
from psychiatric
disorder but some
co-morbidities
may exist

Digestive change / IBS

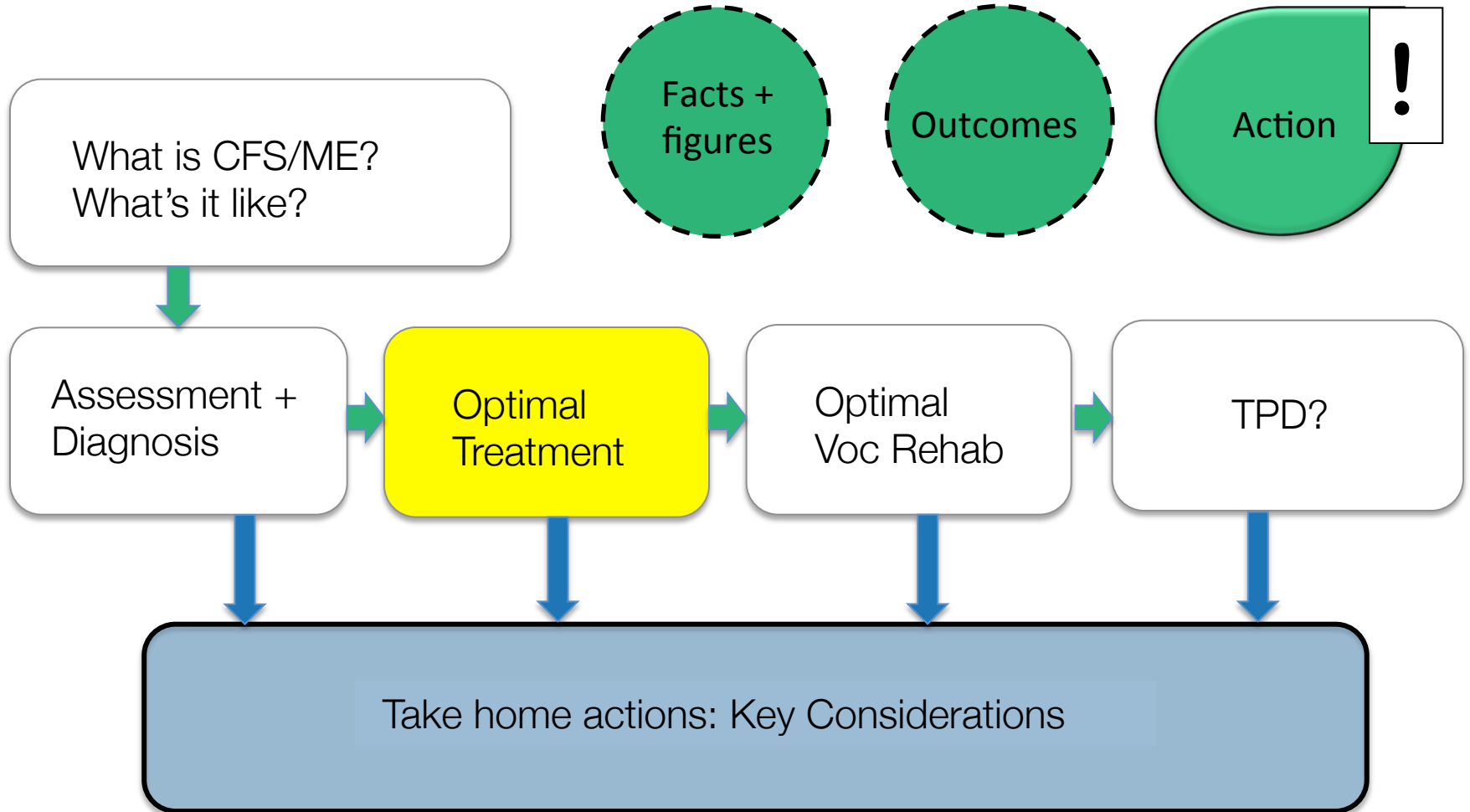
Subjective
temperature changes

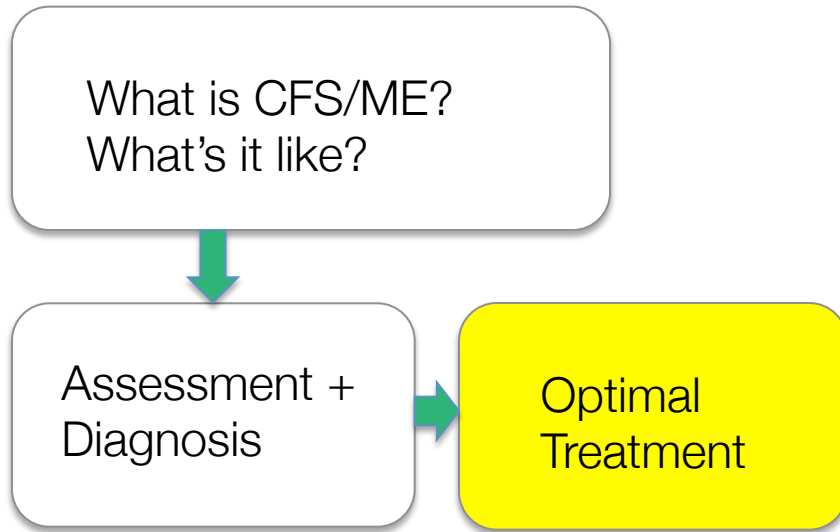


Assessment + Diagnosis:

	Question	+ve	-ve
✓	Thorough biopsychosocial assessment?	Takes more than 8 mins! 1-2 hours subjective examination likely	Didn't consider the full picture, or assessed very quickly or unidimensional
✓	All medical tests as per NICE guidelines (p140+)	Can prove tests have been done + other conditions ruled out	Missing or inconsistent tests, unidimensional testing
✓	Psychiatry (where approp) : to rule out mental health conditions	Liaison psychiatry helped inform the diagnosis, or no sign of MH issue	Emotional health + diagnosis not considered in presence of MH issue
✓	Has sleep apnoea been ruled out, especially in overweight, snoring, males with fatigue?	Specifically assessed and ruled out (sleep tests)	No mention of the possibility in susceptible groups – ie not ruled out
✓	Positive diagnosis	Meets specific positive diagnostic criteria (NICE p145+)	<i>"Well, if there's nothing else wrong it must be CFS/ME"</i>







Effective therapies:

Supported by NICE guidelines + recent research

One-to-

COST
EFFECTIVE

ely structured +
tal:

Exercise Therapy
Behavioural Therapy

?

NHS

Many approaches cause no
change or potentially harmful, eg
Adaptive Pacing, private 'internet'
interventions
THESE DON'T WORK



What are these therapies?

Supported by NICE guidelines + recent research

One-to-One

12-15
sessions

Appropriately
trained
professional

Deliberately structured, goal-
focussed + incremental:

Graded Exercise Therapy:
Gradual, increases physical
activity **BEYOND** current ability
by planning specific exercise

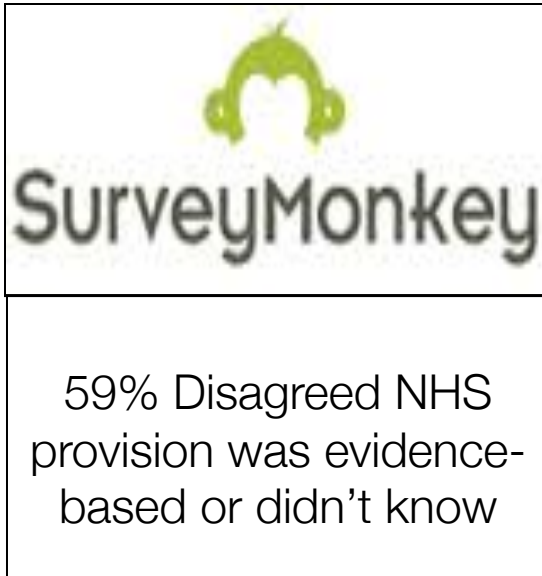
Symptoms
are normal
+ not to be
feared

Stretching
beyond
CAUSES the
+ve change

Long term
change not
quick fix

Cognitive Behavioural Therapy:
Gradual increase activity **BEYOND**
current ability *by addressing fears
+ beliefs*

NHS Treatments?:



"I have had policy holders be told by NHS professionals that they will never RTW"

"I've seen claims were optimum treatment hasn't been considered or followed by the NHS in certain regions of the country"



A flexible service model:



- Home visits
- Office visits
- Clinic space (London)
- Phone
- Skype / FaceTime
- Email



What is NOT recommended?

advertising

lightening products nutritional supplements

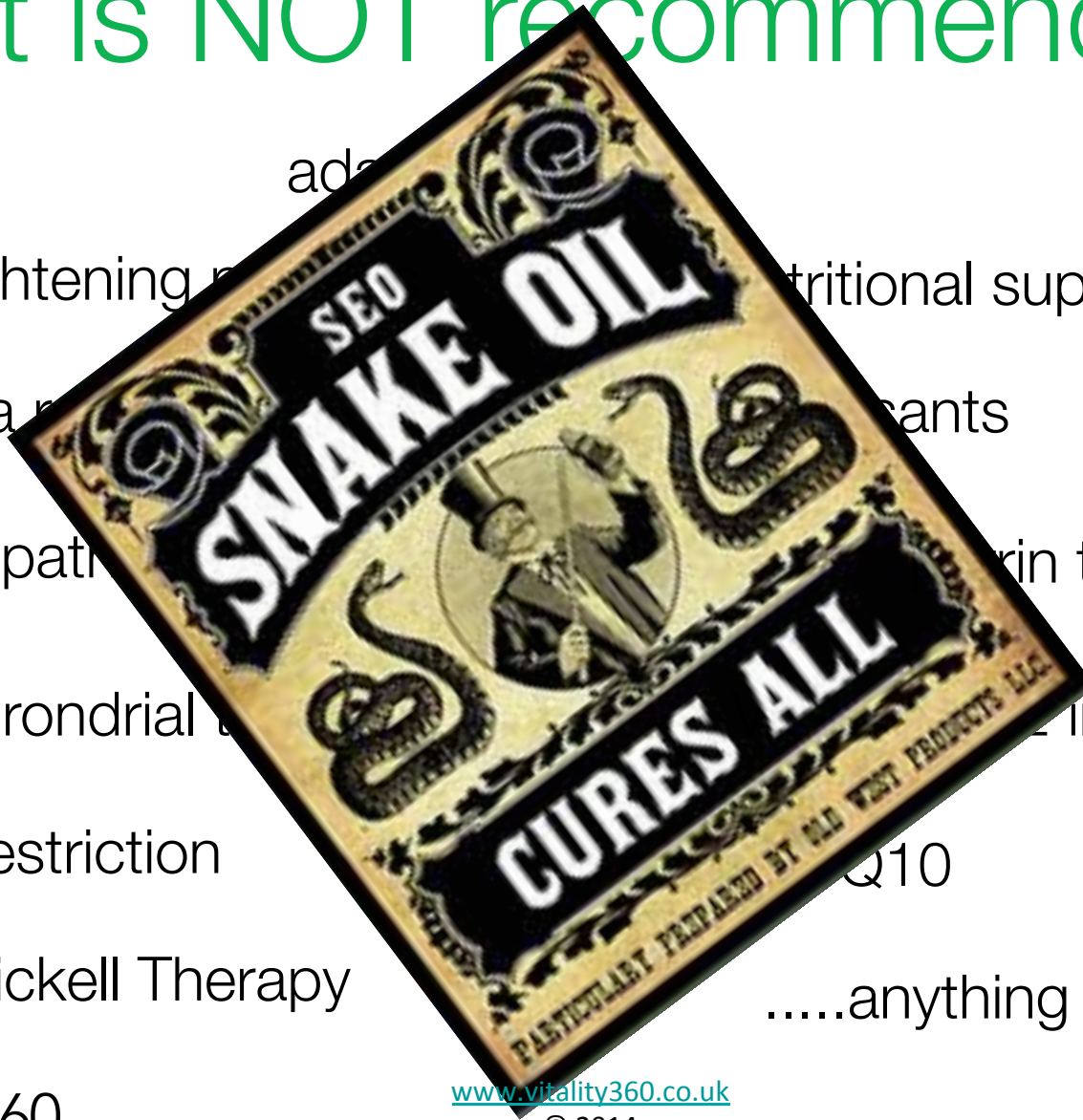
amygdala parents

osteopathy pin technique

mitochondrial injections

dietary restriction Q10

mickell Therapy anything else!!!



A big problem:

- People *think* they have had CBT or GET when they haven't
- *Therapists* think they have delivered CBT or GET and they haven't
- This makes assessing treatment efficacy very difficult for a non-specialist!



OH Guidelines:

“Cognitive behavioural therapy and graded exercise therapy have been shown to be effective in restoring the ability to work in those who are currently absent from work.”

Ref: Occupational Aspects of the Management of Chronic Fatigue Syndrome: a National Guideline (NHS Plus)



What is a treatment programme like?



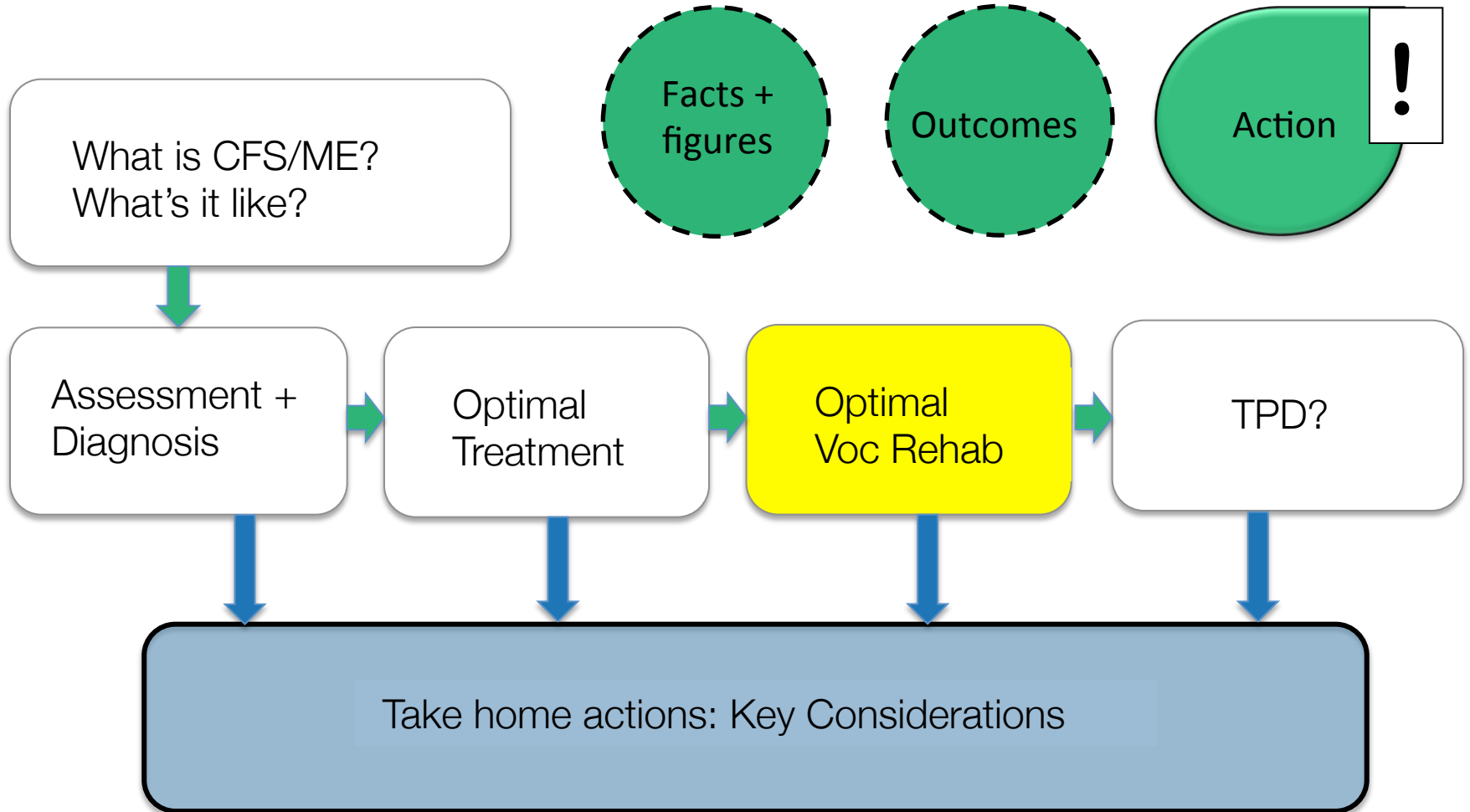
A short video from our
Inspiring People Project



Treatment:

	Question	+ve	-ve
✓	At least 12 sessions of GET or CBT for CFS/ME, delivered 1-2-1?	Therapy provider can prove they have done evidence-based treatment (NICE p188+)	Group sessions, brief, fewer than 12 sessions. Delivering something other than CBT or GET
✓	Therapy message to challenge <i>beyond</i> current capacity? (incremental)	Yes: going beyond is safe, + is what will help get you better	No: Stay within your limits + going beyond = harmful, lack of clarity
✓	Is the therapist appropriately trained?	Therapist can prove training in GET or CBT treatment protocols <i>for CFS/ME specifically</i>	No evidence or insufficient training or supervision, generic CBT or 'exercise' training
✓	Has treatment been compared to best practice by specialist?	Specialist has explored treatment efficacy against evidence-base	Assumption made about treatment efficacy without exploring further
✓	Any additional sleep or mental health issue being treated?	Sleep or MH issue separately treated as well as their CFS/ME	Sleep or MH issue ignored or inadequately treated



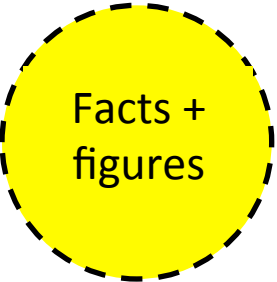


What is the impact of CFS/ME on work?



Work + Economy Stats:

- 50% people in NHS services had discontinued employment due to 'fatigue-related symptoms'
- Estimate of cost to UK economy £100million
- CFS/ME most highly prevalent in working age population (20-40)
- Estimated average benefit figure: per annum £10K



Facts +
figures

*Ref: Insurance industry source
Collin et al 2011*



Poor management

Physical demands

Sitting all day

Bright lights

Lack of understanding

Noise

Poor work culture

Busy office

No rest breaks

Tight deadlines

Emotional workplace

Shift work

Relationship Stress

Complex processing

Back too soon / fast

Long hours

Stigma

High expectations



Adjustments:



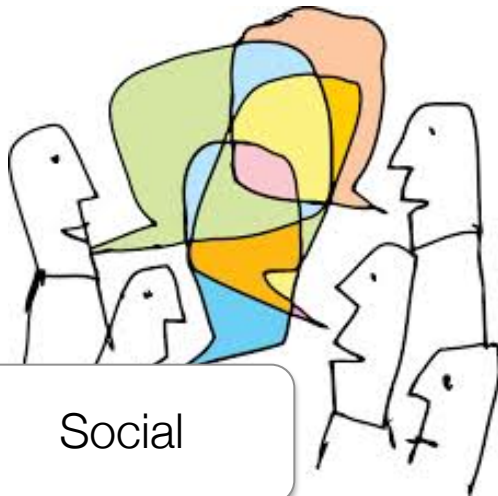
Journey



Lights



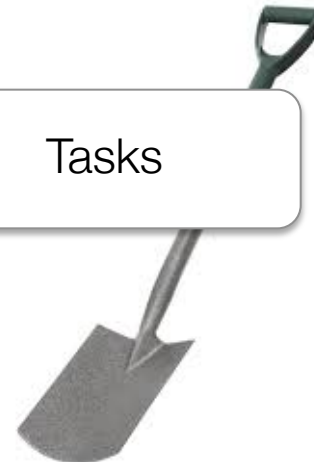
Rest



Social



Cognitive



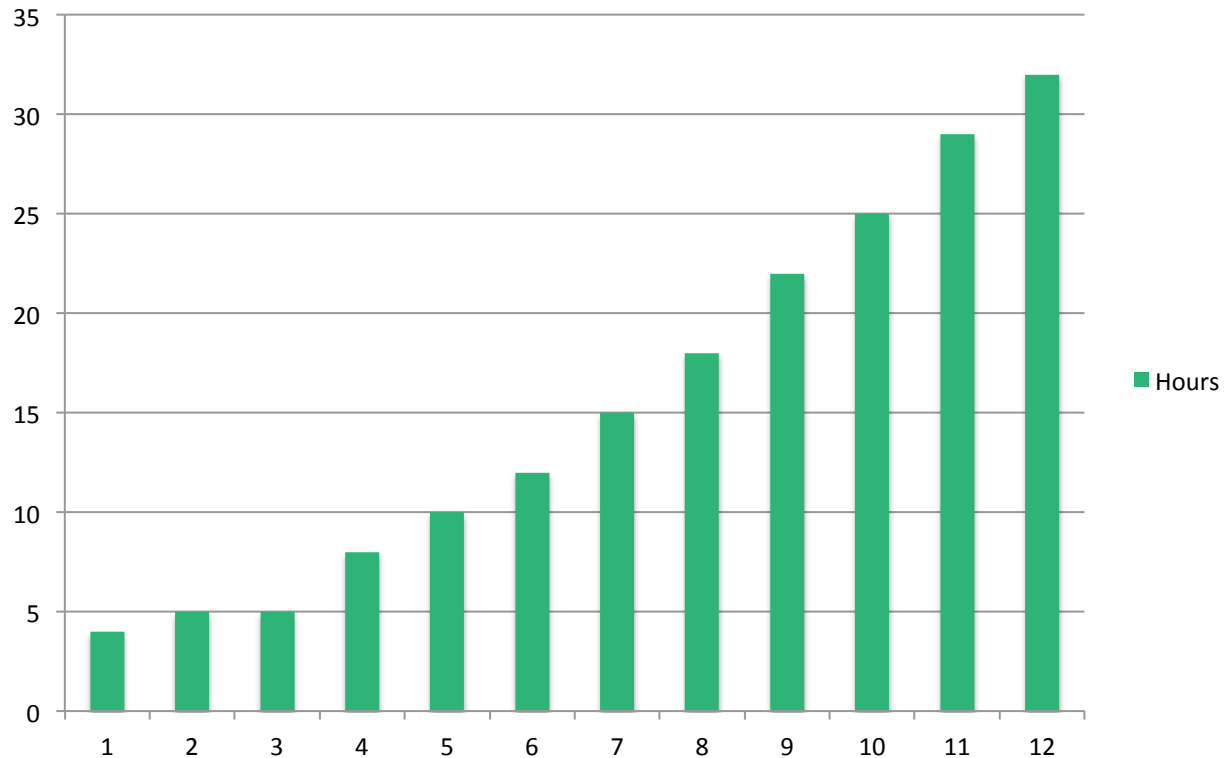
Tasks



Noise

Graded RTW

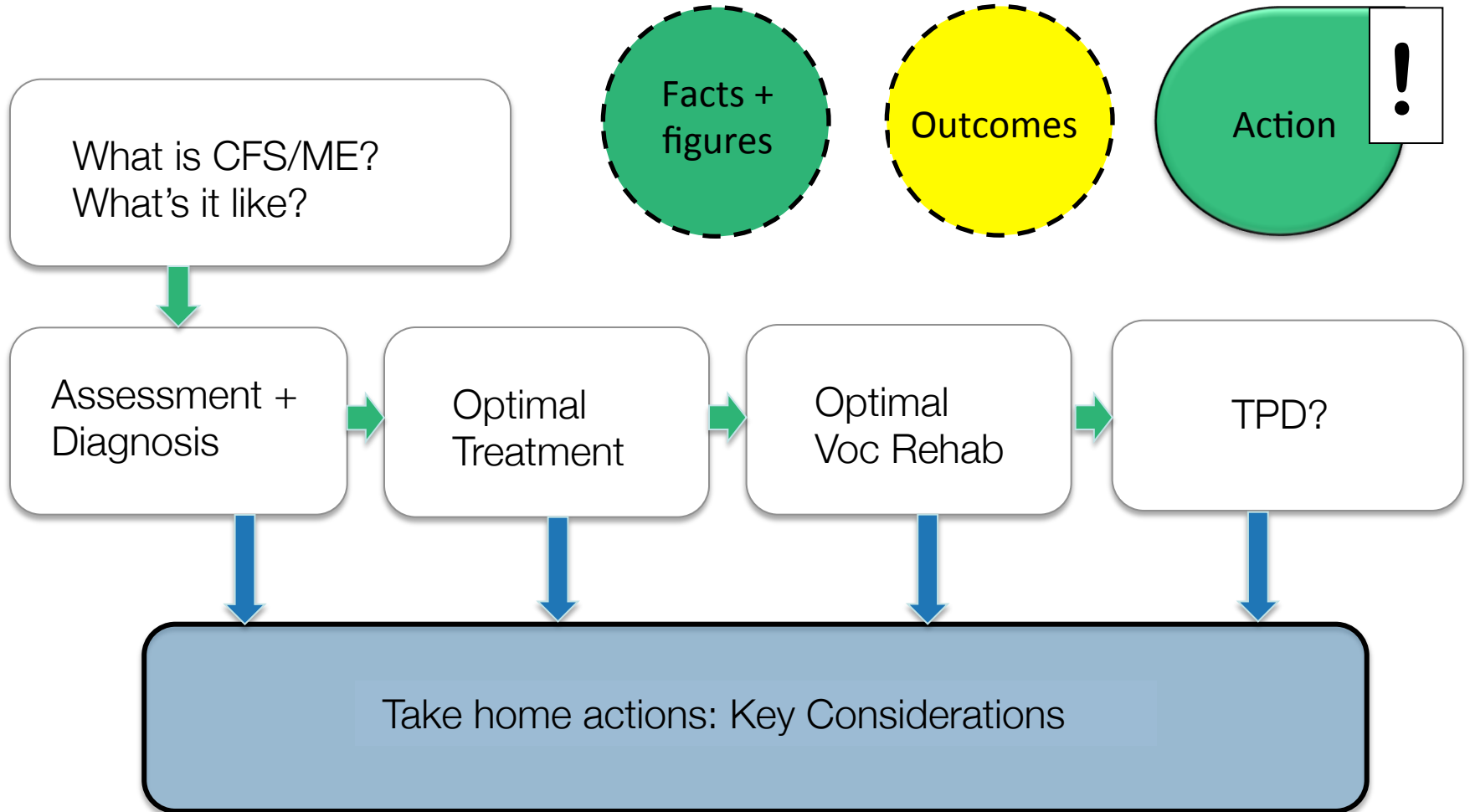
Gradual Increase: Requiring Patience!



Vocational Management:

	Question	+ve	-ve
✓	Is claimant going back to work too soon?	Specialist has advised on timing + baseline of work, structure + role	Pressure to go back too soon, ill considered plan, baseline too high
✓	Have all CFS/ME adjustments been considered?	Include work from home, travel, environment, GRTW, support, rehab	No: Specific CFS/ME adjustments or rehabilitation not been considered
✓	Is the rate of RTW too high? ie steps being made too quickly?	Body adapts to each stage before moving onto next level	Next level instigated before adaptation has occurred > get worse
✓	Has RTW plan been devised with CFS/ME specialist + employer?	Agreement negotiated between all parties to form workable plan	Lack of specialist support or employer input, no or poor plan in place
✓	Is the goal to RTW being proactively managed by therapist?	Vocational issues central to the rehabilitation plan and treatment	Vocational issues not addressed or managed, no focus or attention





Rehabilitation?



RTW Stats:

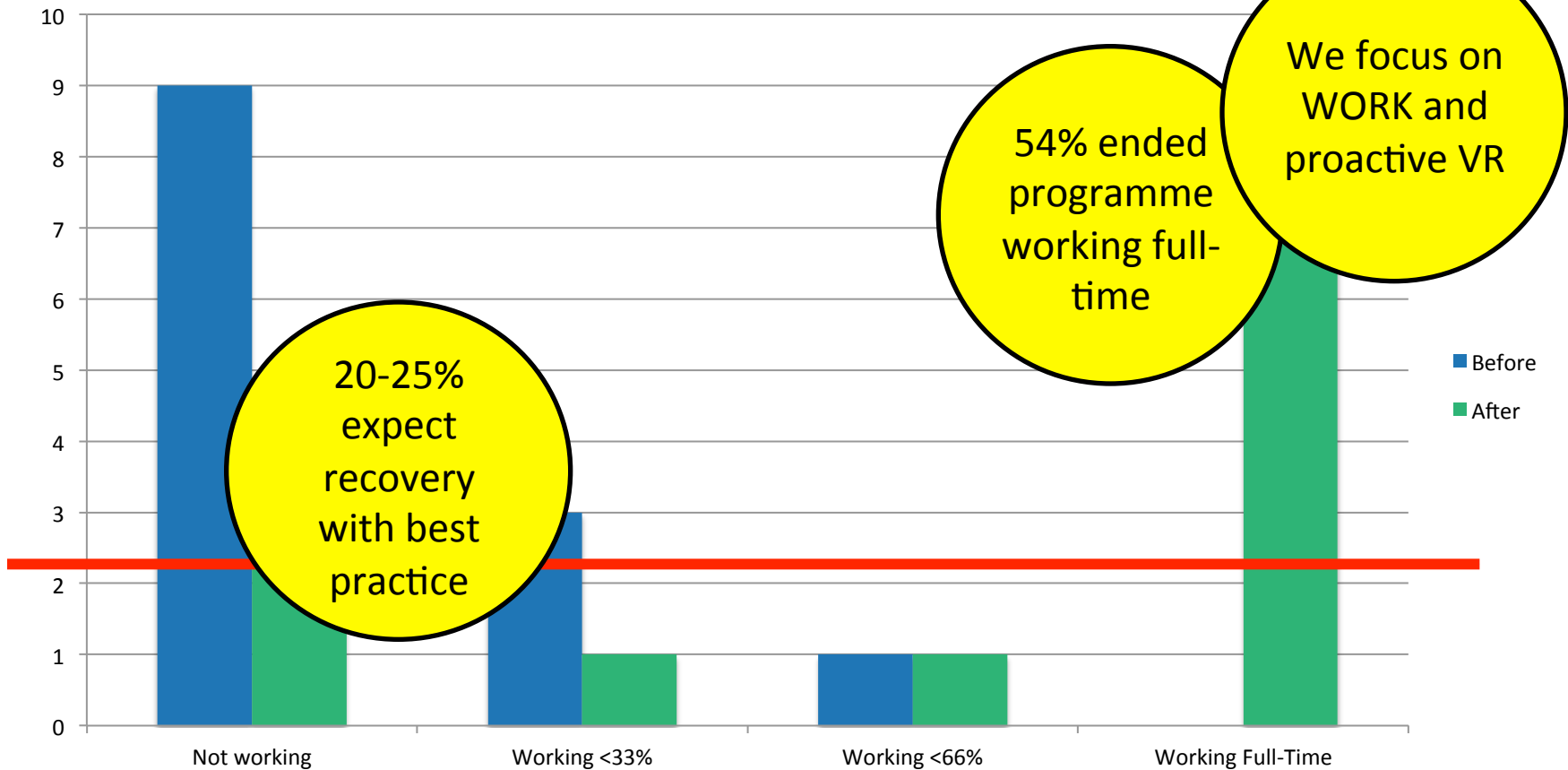
Analysis of Vitality360 CFS/ME database:

- 23 patients recently discharged patients
- Therapy goal included a RTW
 - 43% did not have enough sessions funded to be optimal
 - 57% of these had sufficient sessions (ie at least 12, 1-2-1 sessions) so were analysed



Vitality360 Stats:

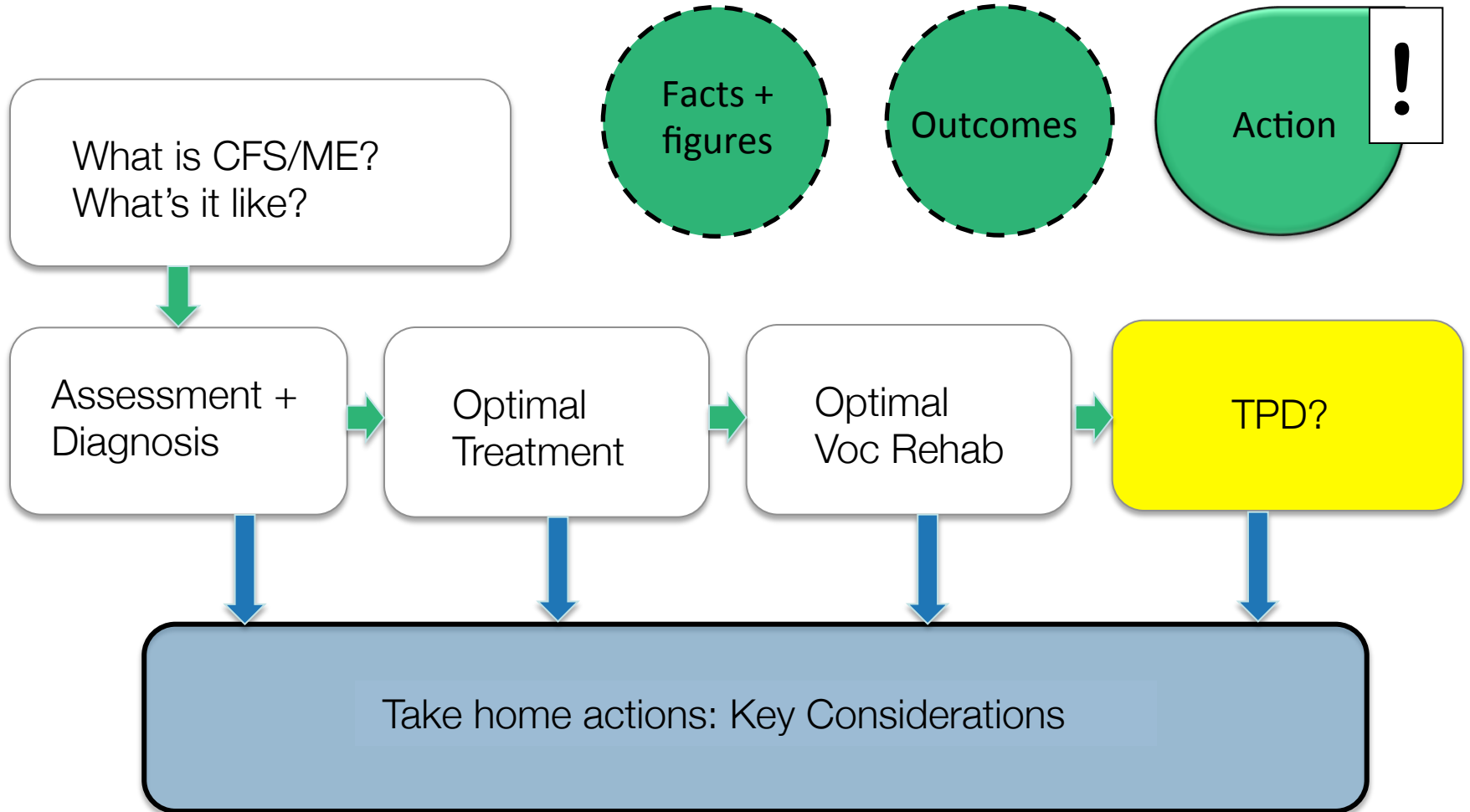
Change in work status before + after Vitality360 Treatment



Benefit of rehabilitation:

“If intervention treatment and management back to work is provided as a comprehensive treatment plan a great many more patients will benefit positively”

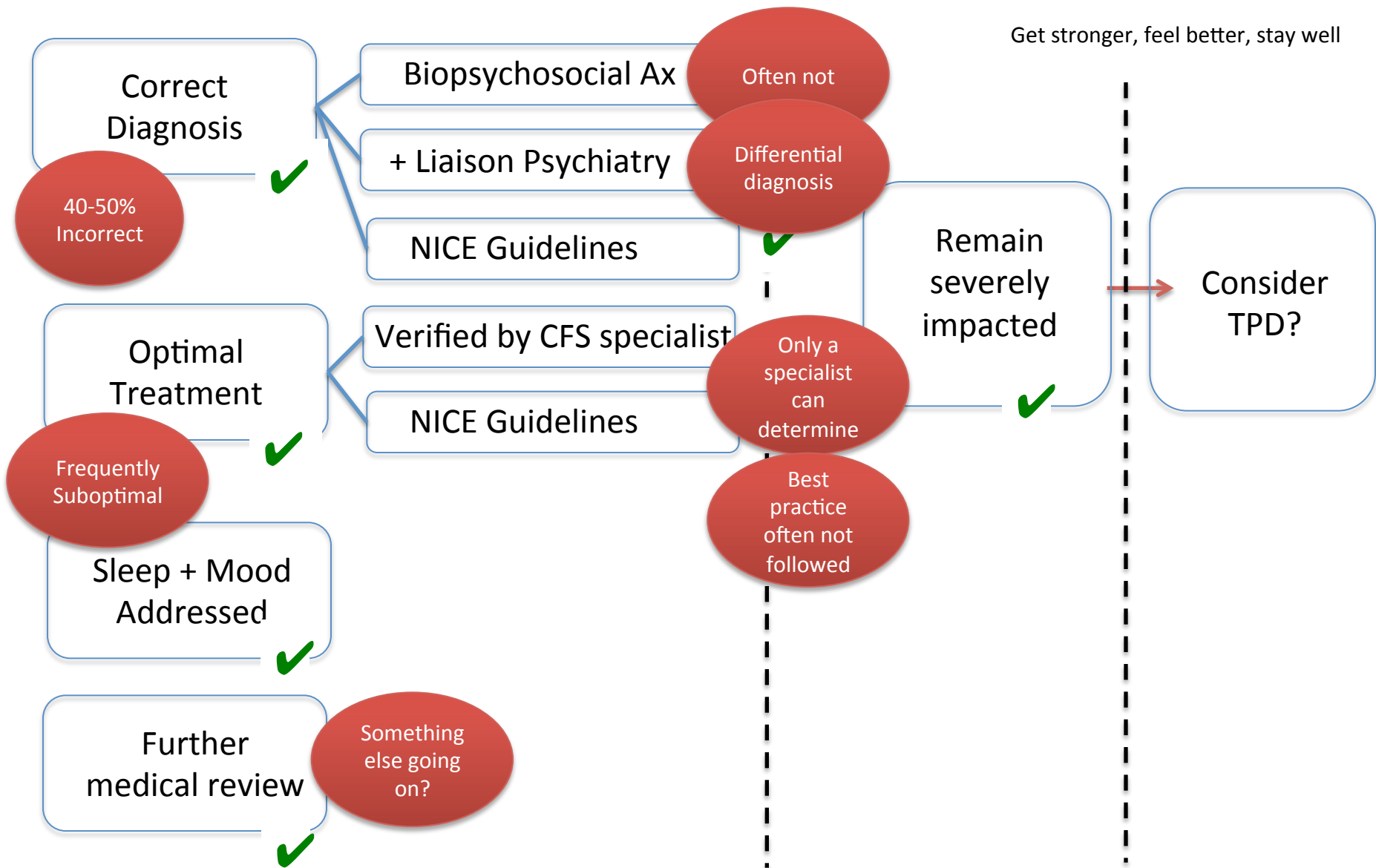




TPD Claims:

- Distilled the various definitions to:





How does CFS/ME impact you?



Survey:

- 106 respondents to *Working with CFS/ME* survey (May 2014)
- 95% from insurance companies
- 79% respondents are involved in financial and therapeutic decisions re: CFS/ME claims
- Most frequent: Respondents had 3-5 claimants with CFS/ME

Ref: NICE Guidelines



Complex

What's really going on?

Challenge!

I will never get them back to work

Here we go again!

Oh no! Long term claim



Some survey stats:

- 93% : Found “Determining whether the claim is genuine + valid” either very or quite challenging
- Most respondents felt only 50% of claims were genuine
- 72% respondents agreed CFS/ME was a genuine + disabling illness
- 53% respondents disagreed that people with CFS/ME are motivated to return to work
- 1 respondent felt 100% of people with CFS/ME could get back to work with the right help



Come and find me for a job!!

Ref: NICE Guidelines



A complex interplay:

*“...all too often the sufferer is well down the line, had poor management, poor support from employers and **just has not had the opportunity** to be managed back to work and will be working with all the positive benefits this brings”*



Let's try to understand

I shouldn't be judgmental

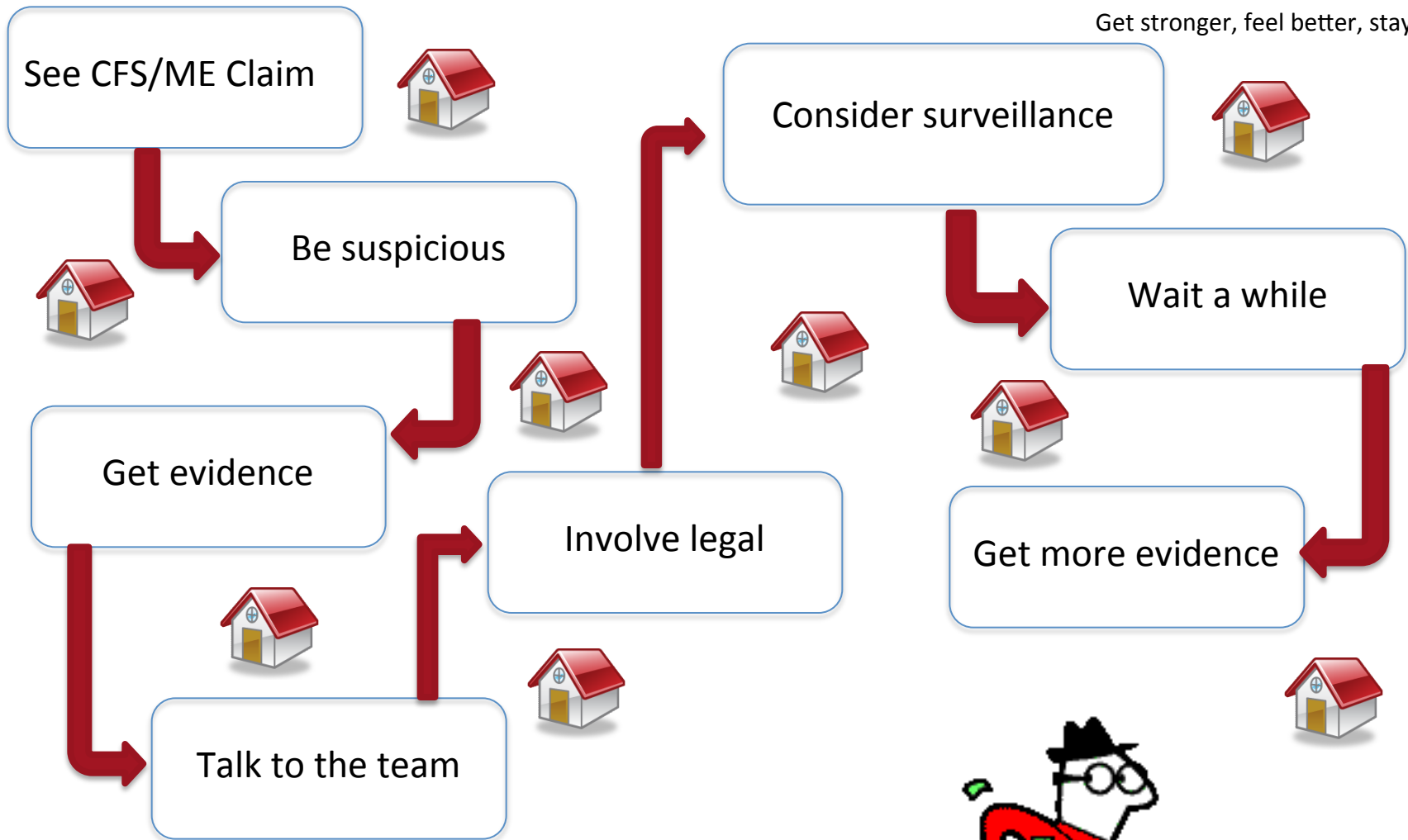
Who made the diagnosis?

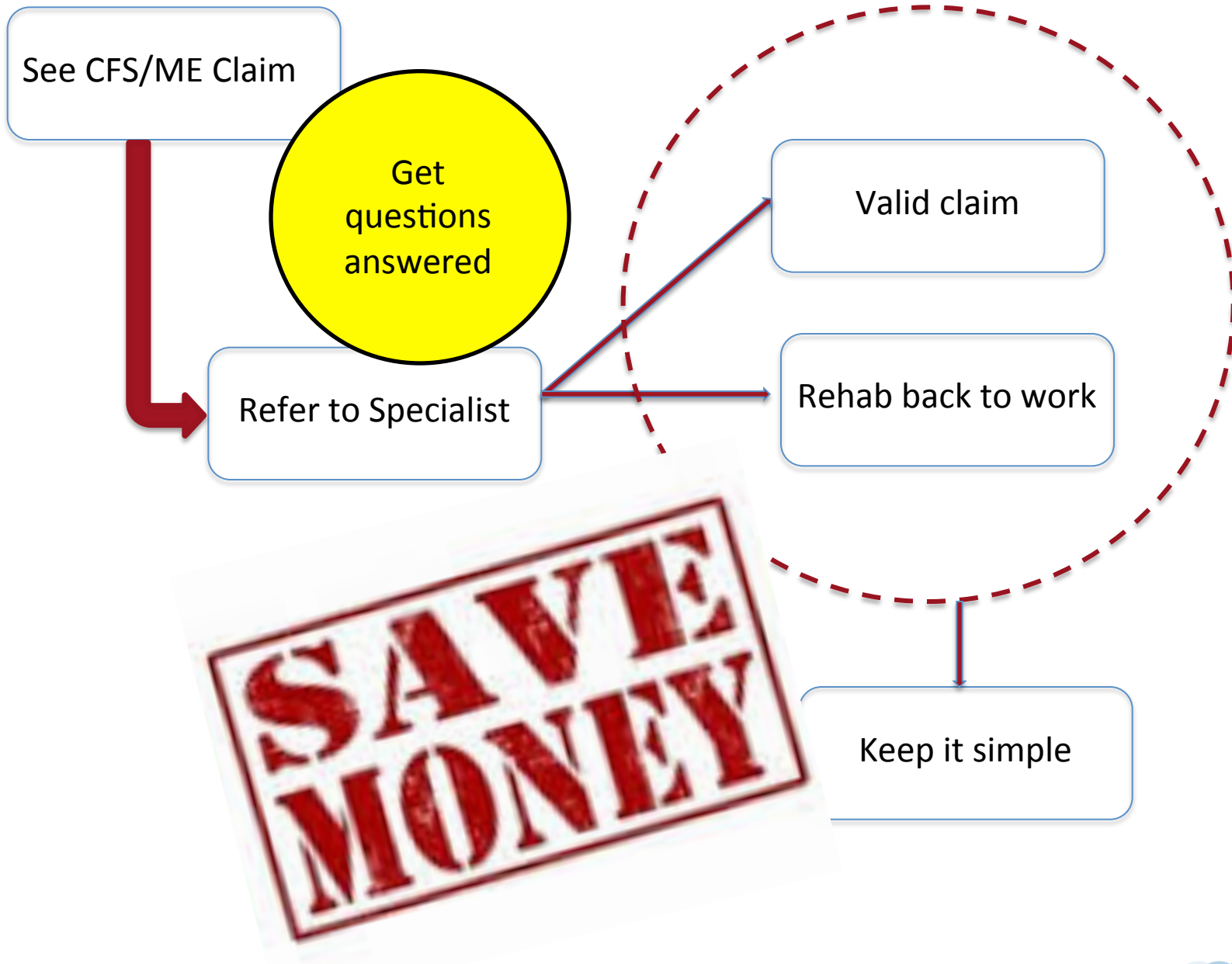
Manage it quickly with appropriate input

What investigations have there been?

BPS factors?





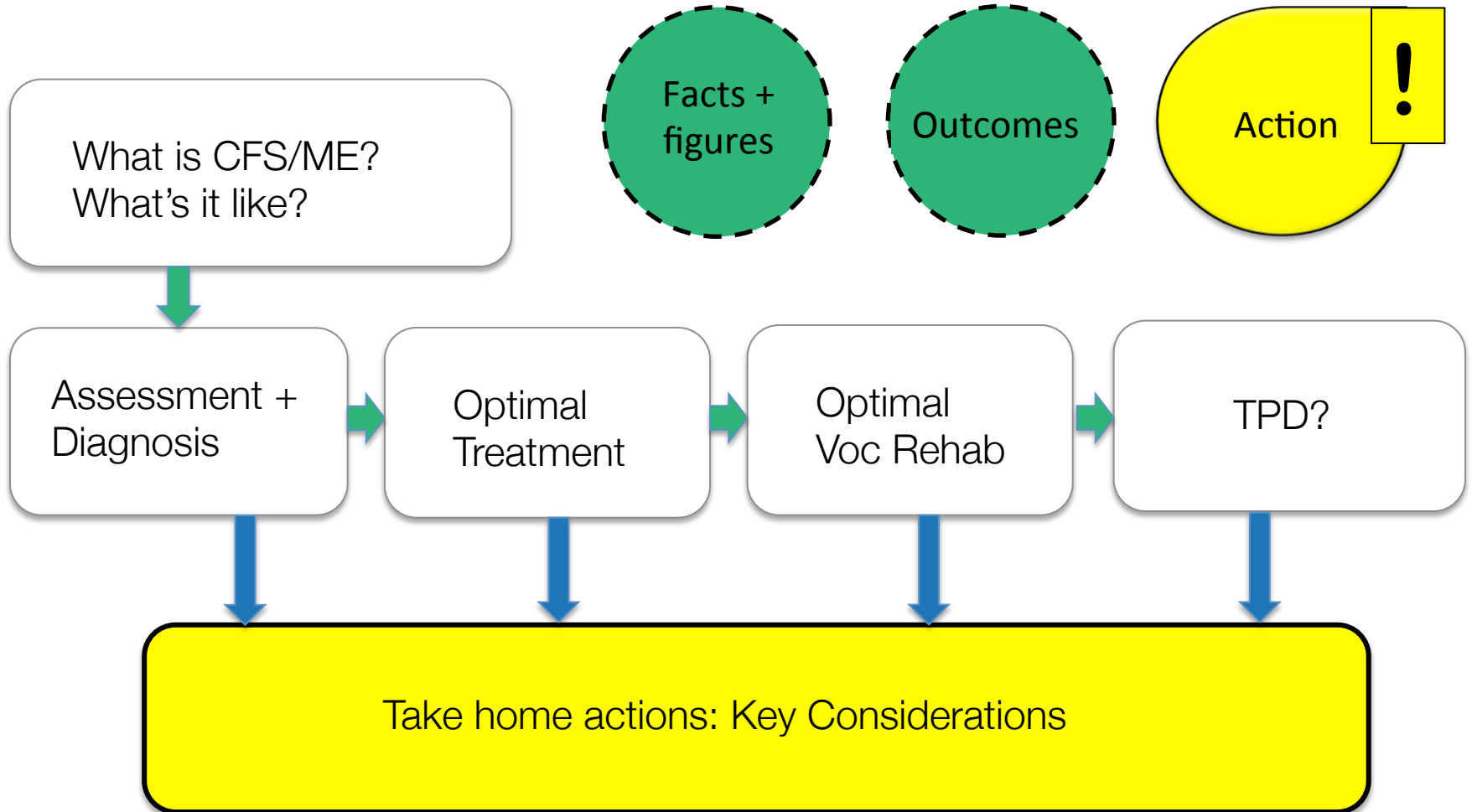


Our commitment:

To create healthy futures and unexpected vitality

This means
proactive
rehabilitation +
supporting
people back to
work





What you can do:

- ✓ Ensure claimants have had the right diagnosis
- ✓ Ensure claimants have had a full biopsychosocial assessment
- ✓ Believe and validate: this is a genuine illness diagnosed by doctors
- ✓ Ensure claimants have had optimal treatment, validated by specialists
- ✓ Ensure sleep and mood have been adequately addressed
- ✓ Refer to specialist CFS/ME teams where diagnosis or treatment is in doubt: but ONLY if you are sure they are offering optimal treatment
- ✓ Intervene early: keep people at work where possible: get help quickly
- ✓ Be prepared to fund treatment where not available on the NHS
- ✓ Be patient with recovery and RTW timescales
- ✓ Be positive about rehabilitation + recovery

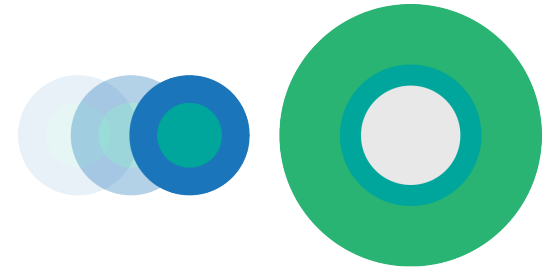


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Thank you

Please do get in touch with any questions:

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To find out more or make a referral, go to:

www.vitality360.co.uk

Get stronger, feel better, stay well